

Name: فائقة امين محمد عبد الله
National ID: 27212021302685



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



DMS

هرموني ومو.ا.ا

Name : Faiqa Amin Muhammad Abdullah

Date : 08-01-2026 09:01

Branch : Nasrcity

Medical No. : 287625

WHOLE BODY ^{18}F -FDG PET/CT STUDY

Clinical History:

- **Patient Age and Gender:** 53 years old female.
- **Clinical Data:** Past history of left breast cancer, previously managed by mastectomy as well as chemotherapy, radiotherapy and hormonal therapy.
- **Pathological Data:** No pathological data available.
- **Previous Study:** No previous PET/CT study available for comparison
- **Study Indication:** PET/CT performed for follow-up.

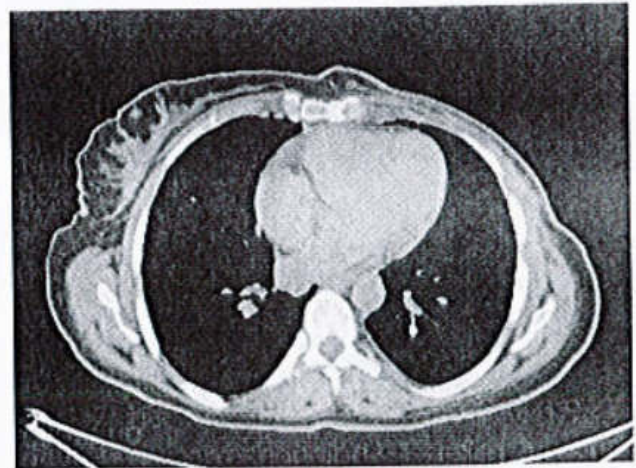
Procedure:

- **Patient Preparation:** Fasted ≥ 6 hours; blood glucose at injection: 90 mg/dL. Weight: 70 kg. Radiopharmaceutical: ^{18}F -FDG, 3.8 mCi I.V., uptake time ~ 60 min.
- **Technique:** Whole-body PET from skull vault to mid-thigh. Corresponding low-dose CT for attenuation correction and anatomic localization and diagnostic CT without iodinated radiocontrast performed. Multiplanar PET, CT, and fused PET/CT reconstructions.
- **Image Quality:** adequate; no significant artifacts or misregistration.
- **Quantitative References:** Liver $\text{SUV}_{\text{max}} \sim 3.6$, $\text{SUV}_{\text{mean}} \sim 2.5$; mediastinal blood pool $\text{SUV}_{\text{mean}} \sim 1.7$; background muscle $\text{SUV}_{\text{mean}} \sim 0.9$.
- A semi-quantitative analysis done, correlated with the corresponding CT series.

Findings:

❖ Loco-regional (Breasts):

- **Primary site:** Left mastectomy operative bed skin thickening, diffuse edema and diffuse low-grade metabolic activity ($\text{SUV}_{\text{max}} \sim 2.2$). Otherwise, no evidence of metabolically active recurrent/residual mass lesion, keeping with post-operative sequelae.
- **Regional lymph nodes:** No FDG-avid regional lymphadenopathy.
- **Local structures:** No evidence of invasion or abnormal metabolic activity in adjacent tissues. No evidence of metabolically active right breast lesion or ipsilateral metabolically active lymphadenopathy.



Call Center **19656**

Name : Faiqa Amin Muhammad Abdullah	Date : 08-01-2026 09:01
Branch : Nasrcity	Medical No. : 287625

❖ Distant:

- **Thoracic:** No FDG-avid pulmonary nodules, pleural deposits, or chest wall lesions.
- **Abdominal/Pelvic:** No abnormal uptake in liver, spleen, adrenal glands, kidneys, or gastrointestinal tract.
- **Skeletal:** No FDG-avid osseous lesions.
- **Other distant sites:** No abnormal uptake suggestive of metastasis.

❖ Other / Incidental Findings:

- **Physiologic uptake:** Normal distribution in brain, salivary glands, myocardium, kidneys, bladder, bowel and bone marrow.
- FDG avid left palatine tonsil, achieving SUVmax~5.9, likely inflammatory.
- Bilateral maxillary and ethmoidal sinusitis.
- Enlarged multi-nodular left thyroid gland with retro-sternal extension, showing diffuse low-grade metabolic activity, achieving SUVmax~2.1; for US correlation.
- Bilateral renal few simple cysts, largest on the right, measuring 5x6 cm.
- Spondylo-degenerative changes of the spine.

Impression:

Post-operative & therapeutic follow-up study, for a reported case of left breast cancer, showing:

- ↓ *Left mastectomy operative bed demonstrates post-operative sequelae, as detailed above. Otherwise, no FDG-avid loco-regional residual or recurrent lesions, lymphadenopathy, or distant metastases are identified. Correlation with sonomammography is advised for comprehensive evaluation.*

Thank you for referral,

Dr. Khaled Zaytoun
Radiologist

Dr. Omnia Talaat, MD
NCI - Cairo University

Patient Name	فايقة امين محمد	Age	52
Sex	F	Special habits	-
Family History	-ve	Comorbidity	Post left MRM post NACT
Surgical History	-	Telephone	01098446167
Menopausal Status	Postmenopausal	Marital Status	married
ID	272120213602685	Medical ID	4225/2024
Date	22-10-2025	Reason for MDT	

Medical Report

Pathology: no residual tumor

IDC ER 7/ 8 PR 8/ 8 her2 : + 3 Ki67 35%

3/2025: Left breast cancer, left retroareolar lesion T4b, N1, IDC G2, received NACT (6x TC and 3 cycles HP only)
MRM in 9/20/2025
ypT0 ypN0 M0:

NACT
Free LN 0/3
PCR

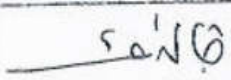
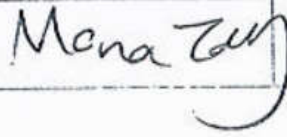
S.M:
Distance from margin : 1 cm away from deep margin.

Metsworkup:

Doctor's notes:

Recommendations:

MDT DECISION	For: • PORT • Continue Phesgo up to 1 year • Ai for 5 years
--------------	---

Name	د/ خالد عبدالعزيز	د/ مني زكي
Signature		

Patient Name	: الأستاذ / فائقه امين محمد عبد الله	Request No.	: 13009297
Patient No.	: 27212021302685	Request Date	: 03/02/2025
Path No.	: SR-201/25	Verification Date	: 18/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass

Nature of Specimen: Slide revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 80% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 35% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with DCIS 30%.

IPT:

- ER: Positive (score 7/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 35% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

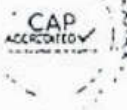
Verified by :

DR.Menna Nabil

Printing Date : 18/03/2025 12:03:59

Page 1 of 2

Printed By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mixr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / فائقة أمين محمد عبد الله	Request No.	: 13002892
Patient No.	: 4225-2024	Request Date	: 30/09/2025
Path No.	: 2979/25	Verification Date	: 11/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 52 Y	Department	: ICU
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer (post NACT) .
Nature of the specimen: Left MRM

Gross Examination:

- Procedure: Left MRM
measured 19x11x5 cm with covering skin ellipse measured 18x8cm & attached axilla 9x6 cm
- Tumor extent : A greyish white tumour bed site
- Size: 5x4x1.5 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 1 cm away from deep margin
- Lymph nodes number and size: 15 L.Ns up to 2.5x1 cm .

BLOCK DESIGNATION :

1:5 TBS

6 .DEEP MARGIN

7. Nipple

8 :10 ALNDS

Grossing by : Dr.Menan Hosam

Microscopic examination:

- Histological type: No residual viable tumor tissue detected
- Ductal carcinoma in situ: No
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/15) with one node showing therapy effect .
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic Complete response

Diagnosis:

Left MRM (post NACT) :

Verified by :

Dr.Heba Hasan



Patient Name : الأستاذة / فائقة أمين محمد عبد الله

Patient No. : 4225-2024

Path No. : 2979/25

Sex : Female

Age : 52 Y

Referring Doc :

Request No. : 13002892

Request Date : 30/09/2025

Verification Date : 11/10/2025

Financial Category : جوستاف روسي - نفقة الدولة

Department : ICU

Registered by : Mrs.Mayada Hegazy

Pathology Report

No residual viable tumor tissue detected.

- Free margins
- Free nipple and areola
- Free L.Ns (0/15)
- Pathologic Complete Response
- Stage : ypT0 , ypN0

Dr. Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Printing Date : 12/10/2025 09:22:10

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Faika Ameen Mohamed Abd Allah	Date : 30/04/2025
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID: 4225/2024	Pat. ID: 95203

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Known case of left breast pathologically proved carcinoma on therapy

No previous film,

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left retro areolar focal asymmetry with increasing density.

Complementary ultrasound :

- Left retro areolar at 12o'clock subtle irregular hypoechoic solid mass lesion with subtle internal vascularity seen within, measuring about 9x7mm.
- No right cystic or solid mass lesions.
- Normal right skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Left multiple pathological axillary lymph node seen at level I,II,III (more than 3nodes) with absent fatty hilum.
- Nonspecific right axillary LN.

Opinion:

- Left retro areolar pathologically proven carcinoma on NACTH BIRADS 6)
- Multiple pathological left axillary LNs seen at level I,II,III.
- Right benign finding (BIRADS 2)

Signature Dr. Nehal Hamdy

30/04/2025 3:08 PM



Patient Name: Falka Ameen Mohamed Abd Allah	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 52 Year(s)
Medical ID: 4225/2024	Pat. ID: 95203

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast conserving surgery.

I. Chest:

- *Left breast parenchymal distortion, no gross circumscribed lesions.*
- *Prominent left axillary level I lymph nodes, the largest measures about 1.2 cm.*
- Two bilateral upper lung lobes pulmonary nodules, the largest measures about 7 mm at right side.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural collection, pleural thickening or masses.
- No cardiac gross enlargement. Minimal pericardial effusion.
- *Enlarged left thyroid lobe showing hypodense nodule measures about 1.6 x 2.4 cm and related calcific focus. For US correlation.*

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- Malroatted both kidneys with their hila facing anterolaterally showing preserved parenchymal thickness. Two bilateral renal cortical simple cysts, largest is at the right lower pole measures about 5.3 x 6 cm.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- Bulky uterus, for US correlation.
- No pathological abdominal or pelvic lymph node enlargement.



Patient Name: Falka Ameen Mohamed Abd Allah	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 52 Year(s)
Medical ID: 4225/2024	Pat. ID: 95203

- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

Reported case of breast cancer revealed:

- Two bilateral upper lung lobes pulmonary nodules. For follow-up.
- No sizable hepatic lesions.

Dr. Ali Elsisy, MSc.

Dr. Mohamed Mohsen, M.D.



Patient Name: Faika Ameen Mohamed AbdAllah	Age: 52 Years
Patient ID: 147684	Exam Date: 13/04/2025
Ref.Phy.Prof. Dr: Mahmoud Abd-Elwahed	

Tc^{99m} MDP DUAL PHASE BONE SCAN:

Procedure:

- Early blood pool and delayed osseous phase whole body scans were obtained 5 minutes and 2 hours respectively post injection of 20 mCi of the tracer.

Previous Imaging:

- No relevant imaging available for comparison.

Findings:

- No abnormal focal active or cold osseous lesions could be detected all over the scanned body to account for serious osseous lesions
- Degenerative changes of spine are seen
- Osteo-arthritic features of both shoulders and knees are also observed
- The early blood pool images were unremarkable with no hypo vascular or hyper vascular areas depicted.

Opinion:

- *The skeleton is free from metastases*

Best Regards,

Prof. Dr. Maha Abd Elkareem

Professor of Nuclear Medicine

Kasr Al Ainy, Cairo University

For Further medical information or if you have any questions please do not hesitate to contact me.

Dr. Ahmed Waheed

01005055422

Dr. M. Nabli El Bolkainy

Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy

Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني

دكتوراه الباثولوجيا - جامعة ميتشيجان

دكتور طارق نبيل البلقيني

دكتوراه الباثولوجيا - جامعة القاهرة

Name: فائقه امين محمد عبد الله

Age: 52 Years Sex: Female

Clinical: Left breast mass

Referred By: Prof

ا.د.

Date: 11-Nov-24

Gross

Fibrofatty tissue pieces measured 4x3x2.5cm.

Microscopic

* The breast tissue shows an invasive carcinoma, composed of nests and cords of cells with ductal differentiation. Tumor cells are large, pleomorphic, cohesive and show mitotic activity. There is associated major intraduct carcinoma component (> 25% of tumor area). This association increases the risk of local recurrence following conservation surgery (24 % in five years).

Diagnosis

Left breast mass: Invasive duct carcinoma, associated with major Intraduct carcinoma component. The tumor is excised with scanty clearance of surrounding normal tissue and it infiltrates close to surgical margins.

Category

Tarek Nabli

M. N. El Bolkainy, M.D. Ph. D.

Date 11/11/2024

{code: 12081 / 0 }

2 Sherif Street, Apt. 18 Cairo. Tel.: 23922022 - Fax : 23927964 فاكس : ٢٣٩٢٢٠٢٢ : ٢٣٩٢٧٩٦٤
٢ شارع شريف - عمارة اللواء - شقة ١٨ - القاهرة - ت : ٢٣٩٢٢٠٢٢ : فاكس : ٢٣٩٢٧٩٦٤
نرجو الاحتفاظ بالشرايح المرفقة كسجل دائم للتشخيص / ويحتفظ بالعينات الجراحية لمدة أسبوع ويلوك الشمع الدال على المرض لمدة عام

جمهورية مصر العربية
بطاقة لتحقق الشخصية

فايقه
امين محمد عبدالله
العدليه
مركز بلبيس - الشرقية

١٩٧٢/١٧/٧٢ ٢٧٢ ١٢ ٠٢ ١٣ ٠٢٦ ٨٥
KX0362851

الكو

٢٠٢٤/١٢ ٢٧٢١٢ ٠٢١٣ ٠٢٦ ٨٥

ربة منزل
أنثى
مسلمة
متزوجة
ايمن حسن حسن سالم ابو الفيل
البطاقة سارية حتى ٢٠٣١/١١/٣٠

٨